

BACK PAIN

Give urgent attention to the patient with back pain and any of:

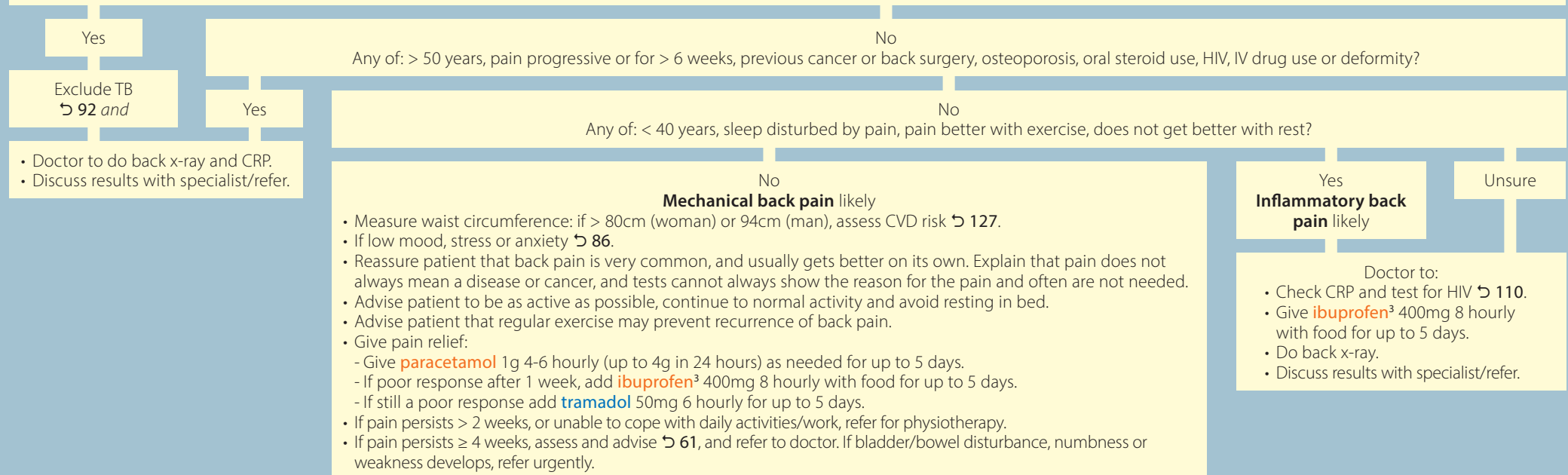
- Bladder or bowel disturbance- retention or incontinence
- Numbness of buttocks, perineum or legs
- Leg weakness or difficulty walking
- Recent injury and x-ray unavailable or abnormal
- Sudden onset severe upper abdominal pain with nausea/vomiting: **pancreatitis** likely
- Pulsatile abdominal mass: **abdominal aortic aneurysm** likely
- If flank, check urine dipstick:
 - If leucocytes/nitrites *with* fever with, and any of: vomiting, BP < 90/60, pulse ≥ 100, diabetes, male, pregnant or post menopause: **complicated pyelonephritis** likely
 - If blood with sudden, severe, one-sided pain radiating to groin: **kidney stone** likely
- Known cancer patient

Manage and refer urgently:

- If **abdominal aortic aneurysm** likely: avoid giving IV fluids even if BP < 90/60 (raising blood pressure may worsen rupture).
- If BP < 90/60 or **pancreatitis** likely, give **sodium chloride 0.9%** 500mL IV over 30 minutes, repeat until systolic BP > 90. Continue 1L 6 hourly. Stop if breathing worsens.
- If **complicated pyelonephritis** likely: first collect urine for MCS and then give **ceftriaxone** 1g IV¹/IM.
- If **kidney stone** likely: give **sodium chloride 0.9%** 1L IV 6 hourly. If pain severe, give **morphine** 10mg IM or 3-10mg slow IV².
- If patient known to have cancer, refer same day.

Approach to patient with back pain not needing urgent attention

- If flank pain with leucocytes/nitrites on urine dipstick, **uncomplicated pyelonephritis** likely: send urine for microscopy, culture, sensitivity. Give **ciprofloxacin** 500mg 12 hourly for 7 days and **paracetamol** 1g 4-6 hourly (up to 4g in 24 hours) as needed for up to 5 days. If no better after 2 days, refer same day.
- Does patient have any of: cough, weight loss, night sweats or fever?



¹Do not mix Ringer's lactate and IV ceftriaxone. Flush IV line with **sodium chloride 0.9%** before and after IV ceftriaxone. ²Dilute 10mg **morphine** with 9mL of sodium chloride 0.9%. Give diluted morphine 3mL IV over 3 minutes (1mL/minute). If needed, give another 1mL/min until pain improved, up to 10mL. Stop if BP drops < 90/60. ³Avoid if peptic ulcer, asthma, hypertension, heart failure or kidney disease. If patient also taking aspirin, advise to wait at least 30 minutes after taking aspirin before taking ibuprofen.